

Product Requirements Document

AI-Driven Prior Authorization System

End-to-End Workflow with Human-in-the-Loop & HIPAA Compliance

Version	1.0
Date	February 2026
Status	Draft
Owner	Product Management
Classification	Confidential
Last Updated	February 9, 2026

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UX/UI Link - <https://claude.ai/public/artifacts/1d693e58-3f38-4c9f-a598-d7fa1f50a131>

1. Executive Summary

1.1 Purpose

This Product Requirements Document (PRD) defines the comprehensive requirements for an AI-driven Prior Authorization (PA) system designed to streamline healthcare authorization processes while maintaining HIPAA compliance and human oversight at critical decision points.

2. Product Overview

2.1 Product Vision

To create the most efficient, accurate, and compliant prior authorization platform in healthcare, leveraging AI to eliminate administrative burden while ensuring clinical appropriateness and patient safety through intelligent human oversight.

2.2 Target Users

User Role	Primary Goals	Key Pain Points
Healthcare Providers	Quick approvals, minimal admin	Long wait times, multiple systems
Clinical Reviewers (RN)	Efficient case review, clear guidance	Manual data gathering, unclear criteria
Medical Directors	Quality oversight, compliance	Inconsistent decisions, audit gaps
Prior Auth Coordinators	Process efficiency, tracking	Manual data entry, status inquiries
Patients/Members	Fast decisions, transparency	Treatment delays, lack of visibility
System Administrators	System reliability, security	Complex integrations, audit requirements

2.3 Key Features Overview

- **Intelligent Document Processing:** OCR, NLP extraction, auto-classification of PA requests
- **AI Clinical Criteria Engine:** Automated matching against MCG/InterQual guidelines
- **Risk-Based Routing:** Smart triage to auto-approve or human review queues
- **Clinical Review Workbench:** Unified interface for RN/MD review with AI recommendations
- **Real-Time Status Portal:** Provider and member self-service tracking
- **HIPAA Compliance Framework:** Built-in encryption, audit logging, access controls
- **Appeals Management:** Automated tracking, deadline management, outcomes analysis
- **Analytics & Reporting:** Dashboards for performance, quality, and compliance metrics

3. Business Requirements

3.1 Business Objectives

- **BR-001:** Reduce PA processing costs by 60% within 12 months of launch
- **BR-002:** Achieve minimum 70% auto-approval rate for routine medical requests
- **BR-003:** Decrease average turnaround time to <24 hours for 80% of requests
- **BR-004:** Improve provider satisfaction scores from 42% to 85%+
- **BR-005:** Reduce appeals rate from 15-20% to <8% through improved accuracy
- **BR-006:** Generate positive ROI within 18 months of deployment
- **BR-007:** Scale to process 1M+ PAs annually without linear cost increase

3.2 Regulatory & Compliance Requirements

- **BR-101:** Achieve and maintain HIPAA compliance for all PHI processing
- **BR-102:** Meet state-specific PA turnaround time requirements (varies by state)
- **BR-103:** Comply with CMS prior authorization requirements for Medicare Advantage
- **BR-104:** Support NCQA accreditation standards for UM programs
- **BR-105:** Enable audit trail for regulatory examinations (DOI, CMS, etc.)
- **BR-106:** Maintain Business Associate Agreements with all vendors
- **BR-107:** Support member appeal rights per state and federal regulations

4. Functional Requirements

4.1 Request Intake & Validation

- **FR-001:** Accept PA requests via multiple channels: EHR integration (HL7/FHIR), secure portal, fax, EDI 278
- **FR-002:** Perform OCR on faxed/scanned documents with 98%+ accuracy for typed text
- **FR-003:** Extract key data elements: patient demographics, diagnosis codes, procedure codes, clinical notes
- **FR-004:** Validate member eligibility in real-time against payer systems
- **FR-005:** Check for duplicate/existing PAs for same patient/service
- **FR-006:** Auto-request missing information with templated outreach to providers
- **FR-007:** Generate unique PA tracking number upon successful intake

4.2 AI Clinical Criteria Matching

- **FR-101:** Match diagnosis and procedure codes against evidence-based medical necessity criteria (MCG, InterQual)
- **FR-102:** Analyze submitted clinical documentation for supporting evidence
- **FR-103:** Check medication requests against formulary and step therapy requirements
- **FR-104:** Identify missing clinical information required for determination
- **FR-105:** Generate confidence score (0-100%) for approval/denial recommendation
- **FR-106:** Provide explainable rationale for AI recommendation with supporting citations
- **FR-107:** Flag edge cases and ambiguous scenarios for human review

4.3 Human-in-the-Loop Review

- **FR-201:** Route cases to appropriate review queue based on complexity and risk score
- **FR-202:** Present AI recommendation with full supporting evidence to clinical reviewer
- **FR-203:** Enable reviewer to accept, modify, or override AI recommendation
- **FR-204:** Require physician review for all proposed denials before finalization
- **FR-205:** Support peer-to-peer consultation requests from treating providers
- **FR-206:** Allow reviewers to annotate cases with clinical rationale
- **FR-207:** Track reviewer agreement/disagreement rates with AI for quality monitoring

4.4 Decision Communication

- **FR-301:** Generate determination letters with clinical rationale and appeal rights
- **FR-302:** Deliver notifications via provider portal, secure email, and postal mail
- **FR-303:** Update EHR systems with authorization details via HL7/FHIR
- **FR-304:** Send member notifications in plain language with next steps
- **FR-305:** Provide alternative treatment options for denials when applicable
- **FR-306:** Include appeal instructions and deadlines in all denial communications
- **FR-307:** Support multi-language notifications (English, Spanish minimum)

4.5 Appeals Management

- **FR-401:** Accept appeals via portal, fax, mail with automated acknowledgment
- **FR-402:** Track regulatory deadlines (30-day standard, 72-hour expedited)
- **FR-403:** Route to independent reviewer (different from initial determination)
- **FR-404:** Provide access to complete original case file for appeal review
- **FR-405:** Support external review process when internal appeals exhausted
- **FR-406:** Analyze appeal outcomes to identify systemic issues and AI improvement opportunities
- **FR-407:** Generate appeal overturn reports for quality monitoring

5. Technical Requirements

5.1 System Architecture

- **TR-001:** Microservices-based architecture for independent scaling and deployment
- **TR-002:** Cloud-native deployment (AWS/Azure/GCP) with multi-region availability
- **TR-003:** API-first design with RESTful and GraphQL endpoints
- **TR-004:** Event-driven architecture for real-time processing and notifications
- **TR-005:** Containerized deployment (Docker/Kubernetes) for portability
- **TR-006:** Service mesh for secure inter-service communication
- **TR-007:** Separate data plane for PHI processing with enhanced security controls

5.2 AI/ML Platform

- **TR-101:** NLP models for clinical document understanding (transformer-based)
- **TR-102:** Classification models for medical necessity determination
- **TR-103:** OCR engine with handwriting recognition capabilities
- **TR-104:** Named entity recognition for extracting ICD-10, CPT, NDC codes
- **TR-105:** Confidence scoring algorithm with calibrated probabilities
- **TR-106:** Model versioning and A/B testing framework
- **TR-107:** Continuous learning pipeline with human feedback incorporation
- **TR-108:** Explainable AI (XAI) framework for transparency and auditability
- **TR-109:** Bias detection and fairness monitoring across demographics

5.3 Data Management

- **TR-201:** HIPAA-compliant data lake for all PA-related data
- **TR-202:** Separate databases for PHI vs. de-identified analytics
- **TR-203:** Data retention policies aligned with state/federal requirements (7-10 years)
- **TR-204:** Automated data backup with point-in-time recovery
- **TR-205:** Data encryption at rest (AES-256) and in transit (TLS 1.3)
- **TR-206:** Data anonymization/de-identification for AI training datasets
- **TR-207:** Master data management for providers, facilities, and payers

5.4 Integration Requirements

- **TR-301:** HL7 v2.x interface for legacy EHR systems
- **TR-302:** FHIR R4 API for modern healthcare interoperability
- **TR-303:** X12 EDI 278 support for payer-to-payer communication
- **TR-304:** Secure FTP/SFTP for batch file exchange
- **TR-305:** Single Sign-On (SSO) via SAML 2.0 and OAuth 2.0
- **TR-306:** Webhooks for real-time event notifications
- **TR-307:** ETL pipelines for claims system integration

6. Non-Functional Requirements

6.1 Performance Requirements

Requirement ID	Metric	Target
NFR-001	API Response Time (p95)	<500ms
NFR-002	Document Processing Time	<2 minutes per document
NFR-003	AI Inference Time	<5 seconds per case
NFR-004	System Availability	99.9% uptime
NFR-005	Concurrent Users	Support 1,000+ simultaneous users
NFR-006	Throughput	Process 10,000+ PAs per day
NFR-007	Database Query Performance	<100ms for 95% of queries
NFR-008	Page Load Time	<3 seconds for web portal

6.2 Security Requirements

- **NFR-101:** Multi-factor authentication (MFA) for all user access
- **NFR-102:** Role-based access control (RBAC) with principle of least privilege
- **NFR-103:** Audit logging of all PHI access with immutable records
- **NFR-104:** Automated session timeout after 15 minutes of inactivity
- **NFR-105:** Encryption of all PHI at rest (AES-256) and in transit (TLS 1.3)
- **NFR-106:** Annual penetration testing and vulnerability assessments
- **NFR-107:** Security Information and Event Management (SIEM) integration
- **NFR-108:** Incident response plan with <1 hour breach notification
- **NFR-109:** Data loss prevention (DLP) controls for PHI
- **NFR-110:** Regular security awareness training for all users

6.3 Scalability Requirements

- **NFR-201:** Horizontal scaling for all stateless services
- **NFR-202:** Auto-scaling based on queue depth and CPU/memory utilization
- **NFR-203:** Database sharding strategy for multi-tenant architecture
- **NFR-204:** CDN for static assets and global performance
- **NFR-205:** Message queue for asynchronous processing (Kafka/RabbitMQ)
- **NFR-206:** Caching strategy (Redis/Memcached) for frequently accessed data
- **NFR-207:** Load balancing across multiple availability zones

6.4 Reliability Requirements

- **NFR-301:** Disaster recovery with RPO <1 hour, RTO <4 hours
- **NFR-302:** Automated failover for critical services
- **NFR-303:** Data replication across multiple geographic regions
- **NFR-304:** Circuit breaker pattern for external service dependencies

- **NFR-305:** Graceful degradation when non-critical services unavailable
- **NFR-306:** Comprehensive error handling and logging
- **NFR-307:** Health check endpoints for all services

6.5 Usability Requirements

- **NFR-401:** WCAG 2.1 Level AA compliance for accessibility
- **NFR-402:** Support for major browsers: Chrome, Firefox, Safari, Edge (latest 2 versions)
- **NFR-403:** Mobile-responsive design for tablet and smartphone access
- **NFR-404:** Maximum 3 clicks to complete common workflows
- **NFR-405:** Inline help and contextual guidance throughout application
- **NFR-406:** Consistent UI/UX patterns across all modules
- **NFR-407:** User satisfaction score >85% in usability testing

7. User Experience & Interface Design

7.1 Design Principles

- **Simplicity:** Minimize cognitive load with clean, intuitive interfaces
- **Efficiency:** Reduce clicks and keystrokes for frequent tasks
- **Transparency:** Make AI recommendations explainable and visible
- **Consistency:** Use standard patterns and terminology throughout
- **Accessibility:** Ensure usability for all abilities and devices
- **Trust:** Build confidence through clear communication and reliability

7.2 Key User Workflows

Provider Portal - Submit PA Request

1. Login with SSO credentials
2. Search for patient by name/ID or scan insurance card
3. Select service/procedure requiring authorization
4. Upload supporting clinical documentation (drag-and-drop)
5. Review auto-populated form with extracted data
6. Submit request and receive confirmation with tracking number
7. View estimated decision timeframe based on case complexity

Clinical Reviewer Workbench - Review Case

1. Login to secure workbench with MFA
2. View prioritized queue sorted by urgency and deadline
3. Click on case to see AI recommendation with confidence score
4. Review patient history, diagnosis, clinical notes in unified view
5. Expand AI rationale section to see supporting evidence and citations
6. Click "View Guidelines" to see applicable medical necessity criteria
7. Make determination: Approve / Deny / Pend for more info
8. For denials: physician counter-signature required before finalization
9. Add clinical notes (auto-saved every 30 seconds)
10. Click "Finalize Decision" to generate determination letter

Member Portal - Check Status

1. Login with member ID and DOB or create account
2. View dashboard with all active and recent PA requests
3. Click on specific PA to see detailed status timeline
4. View determination letter and appeal instructions if applicable
5. Initiate appeal directly from portal if desired
6. Download PDF copies of all communications
7. Set up text/email notifications for status updates

7.3 Wireframe Descriptions

The following wireframe descriptions outline the key screens and UI components. Detailed mockups and prototypes will be developed during the design phase.

Dashboard - Clinical Reviewer

Layout: Three-column responsive design

- **Left Sidebar (20%):** Navigation menu, user profile, quick stats (cases assigned, completed today, overdue)
- **Center Panel (55%):** Case queue with filters (urgency, deadline, type), sortable columns (patient, service, received date, deadline, AI score), color-coded priority indicators
- **Right Panel (25%):** AI insights summary, recent activity feed, compliance alerts, performance metrics

Case Detail View - Clinical Reviewer

- **Header Section:** Patient demographics, insurance info, urgency badge, days remaining until deadline
- **Tab Navigation:** Clinical Summary | Documentation | AI Analysis | Guidelines | History
- **Clinical Summary Tab:** Diagnosis codes with descriptions, requested procedure/medication, relevant vitals and labs, timeline of treatment
- **AI Analysis Tab:** Confidence score (large visual gauge), recommendation (approve/deny), supporting rationale with expandable evidence, conflicting factors if any, similar cases comparison
- **Action Panel (Bottom):** Decision buttons (Approve/Deny/Pend), clinical notes text area, peer-to-peer request button, save draft button

7.4 Visual Design Specifications

Color Palette

Color	Hex Code	Usage
Primary Blue	#1976D2	Primary actions, headers, key UI elements
Secondary Blue	#4FC3F7	Secondary actions, links, highlights
Success Green	#4CAF50	Approvals, positive status, success messages
Warning Orange	#FF9800	Pending items, warnings, attention needed
Error Red	#F44336	Denials, errors, critical alerts
Neutral Gray	#757575	Body text, borders, disabled states

Typography

- **Primary Font:** Inter or Roboto (clean, readable, web-optimized)
- **Headings:** Bold, sizes: H1=24px, H2=20px, H3=16px
- **Body Text:** Regular, 14px, line-height 1.5
- **Small Text:** Regular, 12px for captions and metadata
- **Monospace Font:** Roboto Mono for ICD/CPT codes and technical data

UI Components Library

- **Buttons:** Primary (filled), Secondary (outlined), Text buttons, Icon buttons, Disabled states
- **Forms:** Text inputs, dropdowns, checkboxes, radio buttons, file upload (drag-and-drop), date pickers
- **Cards:** Elevated cards with shadows, Outlined cards, Interactive hover states
- **Tables:** Sortable columns, Filterable rows, Pagination, Row selection, Export functionality
- **Modals:** Confirmation dialogs, Form modals, Full-screen overlays for complex tasks
- **Notifications:** Toast messages (success, info, warning, error), Banner alerts, In-app notification center
- **Progress Indicators:** Linear progress bars, Circular loaders, Step indicators for multi-stage workflows

8. Security & Compliance Requirements

8.1 HIPAA Compliance Controls

HIPAA Safeguard	Control ID	Implementation
Access Control	SC-001	Unique user IDs, MFA, automatic logoff, encryption controls
Audit Controls	SC-002	Immutable audit logs, user attribution, timestamping, log retention
Integrity Controls	SC-003	Data integrity validation, checksums, digital signatures
Transmission Security	SC-004	TLS 1.3 for all transmissions, VPN for remote access
Authentication	SC-005	Multi-factor authentication, password complexity requirements
Encryption	SC-006	AES-256 at rest, TLS 1.3 in transit, key rotation every 90 days
Minimum Necessary	SC-007	RBAC limiting PHI access to job requirements
Workstation Security	SC-008	Screen locks, endpoint encryption, antivirus software
Device Controls	SC-009	Mobile device management (MDM), remote wipe capability
Breach Notification	SC-010	Automated breach detection, notification within 60 days

8.2 Data Privacy Requirements

- **PR-001:** De-identification of PHI for all analytics and AI training datasets
- **PR-002:** Data masking in non-production environments (no real PHI in dev/test)
- **PR-003:** Patient consent tracking for data usage beyond treatment/payment/operations
- **PR-004:** Right to access: members can download their PA history within 30 days
- **PR-005:** Right to rectification: process for correcting inaccurate data
- **PR-006:** Data portability in machine-readable format (JSON/CSV)
- **PR-007:** Privacy impact assessments for new features processing PHI

8.3 AI Governance & Ethics

- **AG-001:** Model transparency: document training data sources, features, and limitations
- **AG-002:** Bias testing across race, age, gender, and geographic demographics
- **AG-003:** Regular fairness audits with statistical parity analysis
- **AG-004:** Human oversight required for all high-risk determinations (denials, high-cost services)
- **AG-005:** Explainability: AI must provide clinical rationale for all recommendations
- **AG-006:** Model drift monitoring with alerts for performance degradation
- **AG-007:** Version control for all AI models with rollback capability
- **AG-008:** Third-party validation of AI accuracy at least annually
- **AG-009:** Clear escalation path for AI errors or unexpected behavior
- **AG-010:** AI decision appeals process separate from standard PA appeals

9. Integration Requirements

9.1 EHR Integration

- **Epic:** HL7 v2.5, FHIR R4, pended orders API, patient context sharing
- **Cerner:** HL7 v2.x, CCL custom scripts for PA status updates
- **Meditech:** HL7 v2.3, custom interface engine configuration
- **Allscripts:** HL7 v2.5, web services API for bidirectional communication
- **Generic FHIR:** FHIR R4 endpoints for any FHIR-enabled EHR

9.2 Payer System Integration

- **INT-201:** Eligibility verification via X12 270/271 or payer API
- **INT-202:** Claims system integration for authorization sync
- **INT-203:** Pharmacy Benefit Manager (PBM) integration for formulary checks
- **INT-204:** Care management system integration for high-risk member identification
- **INT-205:** Provider directory integration for credentialing validation

9.3 External Data Sources

- **INT-301:** MCG Care Guidelines API for evidence-based criteria
- **INT-302:** InterQual criteria engine integration
- **INT-303:** CMS NCD/LCD database for Medicare coverage determinations
- **INT-304:** FDA drug database for medication information
- **INT-305:** NCQA HEDIS measures for quality alignment
- **INT-306:** Social determinants of health (SDOH) data vendors

10. Success Metrics & KPIs

10.1 Operational Metrics

Metric	Definition	Target	Measurement Frequency
Avg. TAT	Time from submission to decision	<24 hrs (routine)	Daily
Auto-Approval Rate	% of PAs auto-approved by AI	>70%	Weekly
Throughput	Total PAs processed per day	10,000+	Daily
SLA Compliance	% of PAs decided within regulatory timeframe	>98%	Daily
Queue Wait Time	Time in human review queue	<4 hours	Hourly
Pended Rate	% of PAs requiring additional info	<15%	Weekly
First Contact Resolution	% resolved without back-and-forth	>80%	Weekly
System Uptime	Availability during business hours	99.9%	Real-time

10.2 Quality Metrics

- KPI-201AI Accuracy: % of AI recommendations matching human reviewer decision (Target: >92%)
- KPI-202Appeal Rate: % of determinations appealed (Target: <8%)
- KPI-203Overturn Rate: % of appeals resulting in reversed decision (Target: <10%)
- KPI-204Inter-Rater Reliability: Agreement between reviewers on same cases (Target: >90%)
- KPI-205Clinical Guideline Adherence: % of decisions aligned with MCG/InterQual (Target: 100%)
- KPI-206Documentation Completeness: % of cases with all required clinical info (Target: >95%)

10.3 User Experience Metrics

- KPI-301Provider Satisfaction (NPS): Net Promoter Score from provider survey (Target: >50)
- KPI-302Member Satisfaction: % satisfied/very satisfied (Target: >85%)
- KPI-303Portal Adoption: % of providers using self-service portal (Target: >75%)
- KPI-304Task Completion Rate: % of workflows completed without errors (Target: >95%)
- KPI-305Time on Task: Average time to complete PA submission (Target: <5 minutes)

- KPI-306Support Ticket Volume: Helpdesk tickets per 1000 PAs (Target: <20)

10.4 Business Impact Metrics

- KPI-401Cost per PA: Total operating cost / # of PAs processed (Target: <\$15)
- KPI-402Cost Savings: Annual savings vs. previous manual process (Target: \$10M+)
- KPI-403FTE Efficiency: PAs processed per FTE (Target: >2,500/month)
- KPI-404Provider Abrasion Reduction: % decrease in provider complaints (Target: >60%)
- KPI-405Revenue Cycle Impact: Days saved in claims processing (Target: 2-3 days)
- KPI-406ROI: Return on investment (Target: Positive ROI within 18 months)

11.5 Go-Live Strategy

- **Pilot Phase:** Launch with 1-2 provider groups and limited PA types, monitor closely
- **Phased Rollout:** Gradually expand to additional providers, geographies, and service types
- **Parallel Run:** Run new system alongside legacy for 30 days to ensure accuracy
- **Training Program:** Comprehensive training for all user roles 2 weeks before go-live
- **Support Model:** Enhanced support during first 90 days with dedicated hotline
- **Communication Plan:** Regular updates to stakeholders, providers, and members

12. Appendices

12.1 Glossary

- **AI:** Artificial Intelligence
- **API:** Application Programming Interface
- **BAA:** Business Associate Agreement
- **CPT:** Current Procedural Terminology
- **EHR:** Electronic Health Record
- **FHIR:** Fast Healthcare Interoperability Resources
- **HIPAA:** Health Insurance Portability and Accountability Act
- **HITL:** Human-in-the-Loop
- **HL7:** Health Level Seven International
- **ICD-10:** International Classification of Diseases, 10th Revision
- **MCG:** Milliman Care Guidelines
- **NLP:** Natural Language Processing
- **OCR:** Optical Character Recognition
- **PA:** Prior Authorization
- **PBM:** Pharmacy Benefit Manager
- **PHI:** Protected Health Information
- **RBAC:** Role-Based Access Control
- **ROI:** Return on Investment
- **SDOH:** Social Determinants of Health
- **SLA:** Service Level Agreement
- **TAT:** Turnaround Time

12.2 Assumptions

- Payers will provide access to eligibility and claims data via API or file exchange
- Providers will have EHR systems capable of HL7 or FHIR integration
- Sufficient volume of historical PA data available for AI model training (100,000+ cases)
- Clinical staff available for model validation and ongoing review
- Regulatory approval obtained for AI-assisted clinical decision making
- Budget approved for third-party clinical criteria licenses (MCG, InterQual)
- Cloud infrastructure costs estimated at \$50,000-\$75,000/month for production
- External vendors (EHR, criteria engines) will meet integration timelines

12.3 Dependencies

- Legal approval of AI governance framework and accountability model
- Completion of HIPAA risk assessment and mitigation plan
- Procurement of clinical criteria licenses (MCG/InterQual)
- Execution of Business Associate Agreements with all vendors
- IT infrastructure readiness (network, cloud accounts, security tools)

- Availability of clinical SMEs for criteria configuration
- Provider and member communication campaigns prior to launch
- Integration readiness from EHR vendors and payers

12.4 Risks & Mitigation

Risk	Impact	Mitigation Strategy
AI model inaccuracy	High	Extensive training data, continuous monitoring, human oversight
Data breach/HIPAA violation	Critical	Multi-layer security, encryption, regular audits, incident response plan
Provider adoption resistance	Medium	Change management, training, clear value demonstration
Integration delays	Medium	Phased approach, backup manual processes, vendor SLAs
Regulatory non-compliance	High	Legal review, compliance officer, external audits
System performance issues	Medium	Load testing, auto-scaling, performance monitoring

12.5 References

- CMS Prior Authorization and Utilization Management Final Rule (2024)
- NCQA Standards for Utilization Management (UM)
- American Medical Association Prior Authorization Physician Survey (2024)
- CAQH Index Report on Healthcare Administrative Costs (2023)
- HIPAA Security Rule Technical Safeguards (45 CFR § 164.312)
- HL7 FHIR Implementation Guide for Prior Authorization
- FDA Guidance on Clinical Decision Support Software
- NIH All of Us Research Program - Health Equity in AI

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